A PSYCHIATRIST'S PERSPECTIVE

How to *Sleep Better*

Integrating modern evidence-based psychiatry with yogic sciences

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By the time you finish reading this sentence, your brain has already begun negotiating your next sleep cycle, whether you like it or not.

THE NIGHT SHIFT

The Night Your Brain Goes to Work

Most people think sleep is the absence of living. It isn't. It is closer to a second shift.

While you lie still, your brain launches into one of the most complex biochemical operations in nature. Memories are filed and cross-referenced. Emotional wounds are quietly processed. Toxic proteins, the kind linked to Alzheimer's disease, are flushed out through a recently discovered glymphatic drainage system that only activates during deep sleep. Miss a night, and that cleaning crew never shows up.

As a psychiatrist, I spend a good part of my day talking to people whose minds have turned against them: anxiety that won't quiet, moods that swing without warning, thoughts that spiral at 3 a.m. And almost without exception, when I ask about sleep, the answer tells me everything.

10x

more likely to have depression with insomnia

17x

more likely to have anxiety with insomnia

That is not a footnote. It is a headline. Yet most people still treat poor sleep as a side effect of stress, not a cause of it.

BOTH DIRECTIONS

The Two-Way Street Nobody Talks About

Sleep and mental health do not have a one-way relationship. They are in a constant, bidirectional conversation. Research from Stanford Medicine's Computational Psychiatry, Neuroscience, and Sleep Laboratory shows, through real-time brain imaging, how a single bad night rewires the emotional centres of the brain.

30%

higher anxiety after just one night of poor sleep (2024, Journal of Neuroscience)

For those managing depression or bipolar disorder, irregular sleep does not just make symptoms harder to bear. It can actively trigger mood episodes.

A 2024 University of Bristol review found that problems with sleep and the internal body clock can trigger or worsen a range of psychiatric disorders, and that understanding an individual's sleep-circadian patterns could unlock earlier, more precise intervention.

The message is uncomfortable but liberating: fix the sleep, and you very often move the needle on the mood.

THE CLOCK

What's Actually Happening When You "Can't Sleep"

Insomnia rarely arrives alone. In clinical experience, it almost always carries a companion: unprocessed anxiety, a nervous system stuck in overdrive, or a lifestyle quietly at war with the brain's ancient rhythms.

Your brain runs on a 24-hour internal clock called the circadian rhythm. This clock regulates not just when you feel sleepy, but when your cortisol peaks, when your body temperature drops, and when melatonin floods in like a tide. Disrupt it through late screens, erratic schedules, or weekend sleep binges, and you are not just tired the next day. You are destabilising one of the most fundamental biological systems you have.

24min

less sleep from one hour of screen time before bed

59%

higher insomnia risk (2024, Journal of Sleep Research)

Your phone is not just keeping your eyes busy. It is sending a false sunrise signal directly to your brain, suppressing melatonin and telling your biology that the day is not over yet.

FIVE MOVES

Evidence-Based Strategies That Work

1 Anchor Your Clock, Every Single Day

Wake up at the same time every morning, including weekends, and get natural light within the first hour. This resets your circadian clock like recalibrating a compass. Over days, your body begins to anticipate sleep at the same hour, making it easier to fall and stay asleep.

> Can't control your wake-up time because of shifts, work, or family? Then anchor the other end. Go to bed at the same time every night. A fixed bedtime gives your body a steady starting point even when your mornings are unpredictable.

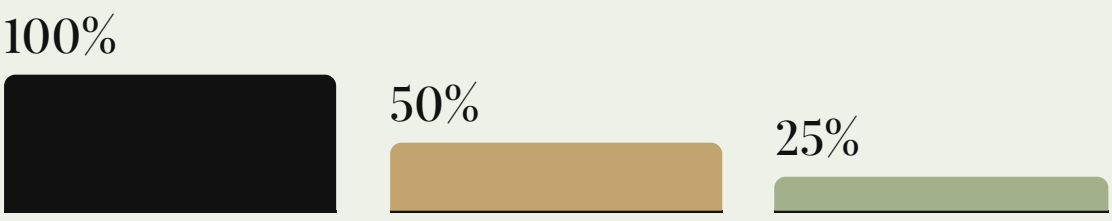
2 Treat the Bedroom Like a Cave

Cool. Dark. Quiet. The brain needs environmental cues to shift into sleep mode. A room that is too warm, too bright, or too noisy keeps the nervous system subtly activated, the biological equivalent of sleeping with one eye open.

3 Cut the Evening Stimulants

Caffeine has a half-life of 5 to 7 hours, so a 4 p.m. coffee is still half-strength in your bloodstream at 10 p.m. Alcohol is more insidious: it may help you fall asleep but fragments the second half of your night, decimating the REM sleep your brain needs for emotional regulation.

A 4 P.M. COFFEE, HOURS LATER



4 p.m.
Full dose

10 p.m.
Half still active

4 a.m.
Quarter still active

Illustration assumes a 6 hour half-life, the midpoint of the 5 to 7 hour range.

4

The 20-Minute Rule

If you are lying in bed and sleep won't come, don't fight it. You cannot force sleep. It is better to let go of the struggle, pick up a book, and read until you are sleepy. When you lie awake frustrated, your brain begins to associate the bed with wakefulness and anxiety, a conditioned response that makes the next night harder.

STRATEGY 5 | THE PSYCHIATRIC GOLD STANDARD

CBT-I

Cognitive Behavioural Therapy for Insomnia (CBT-I) restructures the thoughts and habits that sustain insomnia, often outperforming sleep medication in long-term outcomes.

In the initial phase, medicines may sometimes be needed alongside it. That decision is not one to take alone. It is made together with your psychiatrist.

29
randomised controlled trials

9,475
participants, 2025 meta-analysis

WHY IT MATTERS

Sleep as Mental Health Care

We tend to treat sleep as a lifestyle preference, something to optimise between productivity hacks and morning routines. Psychiatry sees it differently. Sleep problems are common in psychiatric and neurodevelopmental disorders, and often emerge before other symptoms, making sleep a potential early warning signal and a key target for intervention.

Think about that. Before the depression fully arrives. Before the anxiety takes hold. The sleep starts to unravel first. Which means the reverse is also true: protect the sleep, and you are doing something profound. You are protecting the mind before the storm hits.

FROM THE CLINIC

A Final Word From the Clinic

I have seen patients transform, not through dramatic interventions, but through the quiet discipline of taking sleep seriously. Going to bed at the same time. Dimming the phone an hour before bed. Getting outside in the morning light. It sounds ordinary. The results are not.

Your brain is not malfunctioning when it cannot sleep. In most cases, it is responding rationally to an environment, physical and psychological, that is not supporting rest. Change the environment. Change the signals you send your nervous system. Give your brain a reason to power down.

The operating system runs better when it is allowed to reboot.

DON'T JUST COUNT THE HOURS

Quantity is only half the story. Quality matters just as much. Eight hours that leave you groggy are not doing the job. Judge your sleep by how you feel, not only by what the clock says:

- **Refreshed on waking.** You wake up feeling rested, not dragged out of bed.
- **No morning grogginess.** Your head clears within a short time of getting up.
- **No daytime sleepiness.** You get through the next day without fighting to stay awake.

WHEN TO SEEK HELP

If sleep difficulties persist beyond 3 to 4 weeks, or are significantly affecting mood, cognition, or daily function, consult a psychiatrist or sleep specialist. Chronic insomnia is a treatable condition, not a character flaw.

DISCLAIMER This guide is for educational and informational purposes only. It is not medical advice and is not a substitute for professional diagnosis or treatment. Reading it does not create a doctor–patient relationship. Always consult a qualified psychiatrist, sleep specialist or other healthcare professional about your own health, and do not start, stop or change any medication without medical guidance. If you are in crisis or having thoughts of harming yourself, seek emergency help immediately.

ABOUT THE AUTHOR

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Dr Ashish Yadav is a Consultant Neuro-Psychiatrist and internationally certified Yoga Teacher integrating modern evidence-based psychiatry with yogic sciences. His clinical work spans mood disorders, anxiety, insomnia, and the intersection of mind-body medicine. He is committed to making evidence-based mental health care accessible, humane, and grounded in the full spectrum of human experience.

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